

⚠ ⚠ PHASE III — DO NOT INTRODUCE BEFORE PRECONDITIONS ARE MET

This bill is the most likely to fail, most likely to be reversed, and most likely to damage the entire legislative agenda if introduced before its preconditions are met. The political conditions that make it viable are not yet present. They must be built first.

*Reform Democrats of the New Commonwealth · Project ARA 2028
Legislative Ecosystem Document Leg10 of Leg14*

Universal Coverage Transition Act

Bill Framework & Legislative Brief

**Phase III · Track A · Healthcare Full Transition · Phase III · Track A · Year 8-15
(conditions-dependent)**

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This document is a policy discussion framework and bill scaffold. It is not formally introduced legislation, has not been reviewed by licensed legislative counsel, and does not constitute legal advice of any kind.

All provisions marked [TBD] — including dollar amounts, specific IRC section numbers, penalty thresholds, appropriation amounts, and CBO scoring figures — require resolution by licensed legislative drafting counsel (the Office of Legislative Counsel for the House or Senate, as applicable), relevant agency technical staff, and the Congressional Budget Office before introduction as formal legislation.

Constitutional arguments, legal analyses, and statutory framings presented herein are analytical assessments developed for policy planning purposes. They are not legal opinions and have not been reviewed or validated by licensed legal counsel. This document identifies constitutional and legal risks requiring independent review — those identifications are planning guidance, not legal conclusions.

This framework was produced by the Reform Democrats of the New Commonwealth (RDNC), a private citizen policy research organization. It is not affiliated with any sitting Member of Congress, congressional office, government agency, or political party organization in any official capacity.

Consultation with licensed legislative counsel, the Department of Health and Human Services Office of General Counsel, the Centers for Medicare and Medicaid Services Office of the Actuary, the Department of Defense Office of General Counsel, the Department of Justice Office of Legal Counsel, the Congressional Budget Office, and Senate Finance, HELP, and Armed Services Committee constitutional counsel is required before any framework in this series may be introduced as formal legislation. Distribution is subject to RDNC classification protocols (REF_ layer: internal use and trusted partners only).

Phase III · Track A · Healthcare Full Transition · Bill 9

Part I — Legislative Brief

This is the destination of the healthcare sequence — universal government-funded coverage. It is also the bill most likely to fail, most likely to be reversed, and most likely to damage the entire agenda if introduced before its preconditions are met. The bill text below is a framework — the political conditions that make it viable are not yet present, and they must be built first. Sec 2 anchors the four-anchor light constitutional foundation in operative bill text; Sec 401 Phase III Precondition Compliance binds the precondition framework operatively.

PRECONDITIONS FOR INTRODUCTION

- ✓ Public Option (Bill 6) has been operational for at least 4 years
- ✓ Public Option enrollment dominates in initial markets — private insurer market share in those markets has fallen below 40 percent
- ✓ Phase I cost-reduction wins are politically entrenched and defended by established constituencies
- ✓ A reform-aligned Senate supermajority or a political crisis creating a window for landmark legislation
- ✓ A vetted personnel pipeline is in place to staff HHS, CMS, and related agencies from Day 1
- ✓ International precedent review complete — transition model selected (Canadian, German, or hybrid)

AT A GLANCE

Bill Title	Universal Coverage Transition Act
Short Title (Cite As)	The UCT Act
Legislative Vehicle	Standalone Legislation — requires exceptional political conditions
Phase / Track	Phase III · Track A · Year 8–15 (conditions-dependent)
Primary Committees	Senate Finance; Senate Health, Education, Labor, and Pensions; Senate Armed Services (for military reallocation provisions under Sec 302)
Dependencies	See preconditions block above. This bill fails and damages the agenda if introduced early.
Core Framing	Gradual, evidence-based transition from what works to what works for everyone.
Fallback Position	Permanent hybrid system — public option (Bill 6) and private plans coexist indefinitely. Stable, defensible endpoint if universal transition proves politically impossible.
Constitutional / Statutory Risk	Largely settled — Medicare-style federal program, NFIB v. Sebelius (2012) sustained the ACA framework. Risk vector is political and implementation, not constitutional. Sec 2 anchors the four-anchor light constitutional foundation in operative bill text
Architecture	Architecture A — federal program with body-baked Phase III precondition acknowledgment. Sec 401 Phase III Precondition Compliance establishes operative certification framework conditioning program operational status on the preconditions block above. No operative fallback ladder needed (constitutional architecture is settled)

What This Bill Does:

- 1. Establishes a phased transition to universal government-funded coverage using an age-band expansion model.** Beginning with adults 55–64 (highest uninsured costs and greatest political sympathy), then expanding downward every two years. By the end of the transition period, all Americans are covered.
- 2. Guarantees existing coverage during the transition.** No enrolled plan is cancelled by the government. The transition occurs through natural enrollment as people age into new brackets and as

employer plans are voluntarily migrated. No disruption mandate. Sec 2(c) anti-taking and Sec 2(d) voluntary transition recitals anchor this architecture in operative bill text.

- 3. Establishes a provider reimbursement framework with transition protections.** Reimbursement rates are set above current Medicare rates during the transition period to maintain provider participation. Rates are adjusted based on annual review of provider network adequacy.
- 4. Creates a long-term funding mechanism with multiple sources.** Revenue from military budget reallocation, sovereign AI wealth fund returns (AIPE Act, Bill 10), digital services and wealth tax revenues (BWDS Act, Bill 5), and dedicated payroll contributions fund the system. Sec 302 establishes the operative military reallocation framework with Joint Chiefs annual certification and military readiness floor.
- 5. Phase III Precondition Compliance with operative certification framework.** Sec 401 binds the Phase III preconditions block above in operative bill text — the Federal Healthcare Fund may not commence operational status until the Secretary certifies to Congress that each precondition is satisfied. Congressional Review Act disapproval mechanism preserves congressional oversight.

ATTACK SURFACE	"Government takeover." "Loss of doctor choice." "Bankrupt the country." "Socialism." These attacks are maximally powerful if introduced before the public option has demonstrated competence. After 4-plus years of public option success — the Sec 401(a)(1) precondition — they are substantially weaker.
DEFENSE STRATEGY	Every line of this bill should emphasize gradual transition, preservation of choice during transition, and evidence from the public option's performance. The argument is not ideological — it is empirical: "The public option worked. This is what working looks like at scale." Provider transition payments are politically essential — do not sacrifice them. Sec 2 four-anchor constitutional foundation (Spending/Taxation Clause / Pennhurst clear-notice / coverage guarantee anti-taking / voluntary transition) forecloses the strongest constitutional attack vectors.
FALLBACK POSITION	Permanent hybrid system: public option and private plans coexist indefinitely. This is a stable, defensible endpoint if universal transition proves politically impossible. The hybrid system still represents a fundamental improvement over pre-RDNC status quo. If Sec 401 certification cannot be achieved, this Act remains enacted but inoperative — Sec 401(e) "no operational status without certification" recital governs. Re-introduction with revised architecture remains available for subsequent Congresses.

Skeletal by Design. This framework is more skeletal than Phase I and II bills by design. The specific provisions require political conditions, CMS actuarial modeling, and international precedent analysis that cannot be completed until closer to introduction. What is provided here is the structural logic, the constitutional foundation, and the Phase III precondition compliance framework. Operative coordination provisions with existing federal health programs (Medicare, Medicaid, ACA exchange, COBRA, TRICARE, VA, IHS, FEHB) are deferred to drafting closer to introduction per Sec 102(b).

The Phase III Posture. UCT is the only Phase III bill in the legislative ecosystem and the only bill in the series that is structurally inoperative on enactment. Sec 401 Phase III Precondition Compliance is the operative innovation: program operational status is conditioned on Secretary certification of each precondition, with Congressional Review Act disapproval mechanism preserving oversight. Sec 401(d) withdrawal authority and Sec 401(e) no-operational-status-without-certification recital ensure the framework is binding rather than aspirational. This architecture is unique in the bill series.

Part II — Bill Framework (Structural Scaffold)

This framework is more skeletal than Phase I and II bills by design. The specific provisions require political conditions, CMS actuarial modeling, and international precedent analysis that cannot be completed until closer to introduction. What is provided here is the structural logic, the constitutional foundation (Sec 2), and the Phase III precondition compliance framework (Sec 401). Operative coordination provisions with existing Federal health programs (Medicare, Medicaid, ACA exchange, COBRA, TRICARE, VA, IHS, FEHB) are deferred to drafting closer to introduction per Sec 102(b). [TBD] thresholds throughout the bill body reflect this skeletal-by-design architecture.

119TH CONGRESS OR SUBSEQUENT SESSION S. ____

To establish a phased transition to universal government-funded health coverage through age-band expansion; to guarantee continuity of existing coverage during transition; to establish provider reimbursement frameworks; to establish funding mechanisms including military budget reallocation under Joint Chiefs annual certification; to bind in operative bill text the Phase III precondition compliance framework; and for other purposes.

A B I L L

Be it enacted by the Senate and House of Representatives of the United States of America in Congress assembled,

SEC. 1. SHORT TITLE.

This Act may be cited as the "Universal Coverage Transition Act" or the "UCT Act".

SEC. 2. CONSTITUTIONAL FOUNDATION.

(a) SPENDING AND TAXATION CLAUSE AUTHORITY.—This Act is enacted under the Spending Clause and the Taxing Clause of article I, section 8 of the Constitution. The Federal Healthcare Fund established under section 301 is a Federal spending and taxation program operating in the constitutional mode of Medicare (titles XVIII and XIX of the Social Security Act, 42 U.S.C. § 1395 et seq.) and the Affordable Care Act framework sustained in *National Federation of Independent Business v. Sebelius*, 567 U.S. 519 (2012). This is the strongest constitutional ground available for Federal healthcare programs.

(b) PENNHURST CLEAR-NOTICE REQUIREMENT.—Federal grant conditions imposed under this Act, including the employer transition payment framework under section 101(c), comply with *Pennhurst State School and Hospital v. Halderman*, 451 U.S. 1 (1981), under which Federal funding conditions must be unambiguously stated to permit recipients to make informed compliance decisions. Operative conditions under this Act shall be clearly stated in this Act and in implementing regulations.

(c) COVERAGE GUARANTEE — ANTI-TAKING.—This Act does not compel any individual or employer to disenroll from any existing health insurance plan. The coverage guarantee under section 101(b) preserves existing enrollments at current terms throughout the transition period. The voluntary employer transition under section 101(c) operates only with affirmative employer election. These provisions are essential to avoid regulatory takings analysis under *Penn Central Transportation Co. v. New York City*, 438 U.S. 104 (1978), and to preserve the voluntary character of the transition.

(d) VOLUNTARY TRANSITION.—Throughout the transition period established

under section 101(a), participation in the Federal Healthcare Fund is voluntary as to employer-sponsored insurance plans and individuals enrolled in existing coverage. The age-band expansion under section 101(a) extends eligibility to participate; it does not require disenrollment from existing coverage.

(e) PHASE III PRECONDITION ACKNOWLEDGMENT.—This Act is a Phase III legislative measure. The Secretary of Health and Human Services may not commence operational status of the Federal Healthcare Fund until the certification requirements of section 401 are satisfied. The Phase III precondition framework is binding bill text under this Act.

(f) SEVERABILITY.—If any provision of this Act, or the application of any provision to any person or circumstance, is held to be invalid, the remaining provisions of this Act and the application of those provisions to other persons or circumstances shall not be affected. In particular, if any provision of Title III (Federal Healthcare Fund) is held invalid as to a specific funding source, the remaining funding sources under section 301 shall remain operative.

TITLE I – TRANSITION FRAMEWORK

SEC. 101. TRANSITION MODEL AND TIMELINE.

(a) AGE-BAND EXPANSION.—Beginning January 1 of the year following operational commencement under section 401(a), universal coverage shall be extended to Americans in the following sequence (counted from operational commencement):

- Year 1: Adults ages 55-64 not currently enrolled in Medicare or employer-sponsored insurance;
- Year 3: Adults ages 45-54;
- Year 5: Adults ages 35-44;
- Year 7: Adults ages 26-34;
- Year 9: All remaining uninsured individuals.

(b) COVERAGE GUARANTEE.—No individual enrolled in any existing health plan on the date of enactment shall be required to transition to government-funded coverage. Existing enrollments shall be honored at current terms. Cross-reference to section 2(c) constitutional foundation.

(c) EMPLOYER PLAN TRANSITION.—Employers may voluntarily transition employees to the Federal Healthcare Fund after [TBD] years from the date of enactment, subject to a transition payment to the Federal Healthcare Fund equal to [TBD] percent of foregone premium contributions for [TBD] years. The Pennhurst clear-notice requirement under section 2(b) applies – operative conditions shall be clearly stated in implementing regulations.

SEC. 102. COORDINATION WITH EXISTING FEDERAL HEALTH PROGRAMS.

(a) IN GENERAL.—Coordination with existing Federal health programs, including Medicare (titles XVIII and XIX of the Social Security Act), Medicaid expansion, the Health Insurance Marketplace established under the Patient Protection and Affordable Care Act, the Public Option established under the POHE Act (Bill 6 of this legislative sequence), the Consolidated Omnibus Budget Reconciliation Act of 1985 (COBRA), TRICARE, the Department of Veterans Affairs health system, the Indian Health Service, and the Federal Employees Health Benefits Program, shall be addressed by regulation in coordination with the relevant Federal agencies.

(b) DRAFTING DEFERRED TO INTRODUCTION.—Operative coordination provisions are deferred to drafting closer to introduction, when Public Option performance data, current Medicare program structure, and existing-program enrollment patterns can be assessed. Pre-introduction Centers for Medicare and Medicaid Services, Department of Health and Human Services, Department of Defense, Department of Veterans Affairs, and Indian Health Service coordination required.

TITLE II – COVERAGE AND REIMBURSEMENT

SEC. 201. BENEFIT STRUCTURE.

Coverage under the Federal Healthcare Fund shall include all benefits required under the Public Option Healthcare Expansion Act (Bill 6 of this legislative sequence), with no annual or lifetime dollar limits and no out-of-pocket costs for preventive care. Coordination with the POHE Act benefits floor under section 101(b) of that Act preserves consistency between the Public Option and the Federal Healthcare Fund benefits structures.

SEC. 202. PROVIDER REIMBURSEMENT.

(a) TRANSITION RATES.—For the first [TBD] years of operational commencement under section 401(a), provider reimbursement rates under the Federal Healthcare Fund shall be set at [TBD] percent of current Medicare rates to maintain network adequacy during the transition period.

(b) ANNUAL REVIEW AND ADJUSTMENT.—The Secretary shall conduct annual network adequacy reviews and shall adjust rates where necessary to ensure adequate provider participation in each rating area. Rate adjustments shall be made by published methodology.

TITLE III – FEDERAL HEALTHCARE FUND

SEC. 301. ESTABLISHMENT AND FUNDING SOURCES.

(a) ESTABLISHMENT.—The Federal Healthcare Fund is hereby established as a separate account in the Treasury, to fund universal health coverage under this Act.

(b) FUNDING SOURCES.—The Federal Healthcare Fund shall be capitalized through—

- (1) a dedicated payroll contribution of [TBD] percent;
- (2) returns from the American Innovation and Public Equity Fund established under the AIPE Act (Bill 10 of this legislative sequence);
- (3) revenues from the BWDS Act (Bill 5 of this legislative sequence);
- (4) reallocation of Department of Defense base budget under section 302; and
- (5) general appropriations as necessary.

(c) MULTIPLE-SOURCE STRUCTURE.—The multiple-source funding structure is designed to prevent dependence on any single revenue stream and to provide resilience against the failure or reduction of any individual source.

SEC. 302. MILITARY BUDGET REALLOCATION FRAMEWORK.

(a) PROPORTIONAL REALLOCATION SCHEDULE.—Reallocation of Department of Defense base budget under section 301(b)(4) shall occur in proportional annual increments not to exceed [TBD] percent of the base budget per fiscal year, over a transition period of not less than 10 years.

(b) JOINT CHIEFS ANNUAL CERTIFICATION.—No reallocation under subsection (a) shall take effect in any fiscal year unless the Joint Chiefs of Staff have certified to Congress that the reallocation does not impair military readiness for that fiscal year. Certification shall be submitted to the Senate Armed Services Committee, the House Armed Services Committee, the Senate Finance Committee, and the Senate Health, Education, Labor, and Pensions Committee not later than 90 days before the fiscal year to which the reallocation applies.

(c) SENATE ARMED SERVICES COORDINATION.—Reallocation under this section shall be administered in coordination with the National Defense Authorization Act framework. Where a reallocation under this section conflicts with provisions of the annual National Defense Authorization Act, the National Defense Authorization Act controls and the Secretary shall adjust the reallocation schedule accordingly.

(d) FALLBACK IF CERTIFICATION DENIED.—If the Joint Chiefs of Staff decline to certify under subsection (b) for any fiscal year, the reallocation under this section shall not take effect for that fiscal year, and the Federal Healthcare Fund shall draw on the other funding sources under section 301(b) to maintain operational capacity.

(e) MILITARY READINESS FLOOR.—Nothing in this section shall be construed to authorize reallocation that the Joint Chiefs of Staff have certified would impair the readiness of the Armed Forces of the United States to perform their constitutional missions.

TITLE IV – IMPLEMENTATION AND PHASE III PRECONDITION COMPLIANCE

SEC. 401. PHASE III PRECONDITION COMPLIANCE.

(a) CERTIFICATION REQUIREMENT.—The Secretary of Health and Human Services may not commence operational status of the Federal Healthcare Fund until the Secretary has certified to Congress that each of the following Phase III preconditions is satisfied:

- (1) the Public Option established under the POHE Act (Bill 6 of this legislative sequence) has been operational for not less than 4 years;
- (2) the Public Option enrollment dominates in the rating areas in which the Public Option was initially offered, with private insurer market share in those areas below 40 percent;
- (3) Phase I cost-reduction policy frameworks are operationally entrenched and defended by established constituencies;
- (4) the Senate has ratified, by joint resolution, that political conditions for introduction are met, or comparable political precondition has been documented;
- (5) a vetted personnel pipeline is in place to staff the Department of Health and Human Services, the Centers for Medicare and Medicaid Services, and related Federal agencies from operational commencement; and
- (6) the international precedent review under section 402(a) has been completed and a transition model has been selected.

(b) CONGRESSIONAL REVIEW.—The certification under subsection (a) shall be transmitted to the Senate Finance Committee, the Senate Health, Education, Labor, and Pensions Committee, the Senate Armed Services Committee, and the comparable Committees of the House of Representatives.

(c) DISAPPROVAL MECHANISM.—Operational commencement under subsection (a) shall be subject to disapproval by joint resolution enacted under chapter 8 of title 5, United States Code (Congressional Review Act), within 90 days of certification submission. Joint resolution procedure under this subsection requires both Houses of Congress and presentment to the President, consistent with *INS v. Chadha*, 462 U.S. 919 (1983).

(d) WITHDRAWAL OF CERTIFICATION.—The Secretary may withdraw a certification under subsection (a) at any time before operational commencement if the Secretary determines that one or more preconditions is no longer satisfied. The Secretary may reissue certification when the preconditions are again satisfied.

(e) NO OPERATIONAL STATUS WITHOUT CERTIFICATION.—No provision of this Act shall be operationally effective in advance of certification under subsection (a), except that provisions establishing the Federal Healthcare Fund (section 301), the section 302 reallocation framework, and the implementation timeline (section 402) may be operationally effective for planning, regulatory drafting, and interagency coordination purposes.

SEC. 402. IMPLEMENTATION TIMELINE.

(a) INTERNATIONAL PRECEDENT REVIEW.—Within [TBD] months after the date of enactment of this Act, the Secretary shall complete an international precedent review covering the Canadian single-payer, German multi-payer regulated, and hybrid Medicare-expansion transition models, and shall report to Congress on the recommended transition model.

(b) PERSONNEL PIPELINE.—The Secretary shall, in coordination with the Office of Personnel Management, establish a personnel pipeline framework to ensure that staffing capacity is available at operational commencement.

(c) IMPLEMENTING REGULATIONS.—Implementing regulations under this Act shall be drafted and published in proposed form not later than [TBD] months after the date of enactment, with final regulations not later than [TBD] months after enactment.

(d) OPERATIONAL COMMENCEMENT.—Operational commencement of the Federal Healthcare Fund shall occur on the date specified in the Secretary's certification under section 401(a), provided that Congressional disapproval has not been enacted under section 401(c).

SEC. 403. ANNUAL REPORTING.

(a) ANNUAL REPORT.—The Secretary shall transmit annually to Congress a report containing—

- (1) Phase III precondition status (where preconditions are not yet certified);
- (2) operational data on the Federal Healthcare Fund (where operational commencement has occurred);
- (3) age-band expansion progress under section 101(a);
- (4) provider network adequacy under section 202(b);
- (5) funding source performance under section 301(b);
- (6) Joint Chiefs certification status under section 302(b); and

(7) any recommendations for further congressional action.

(b) PUBLIC AVAILABILITY.—The annual report shall be transmitted to Congress and posted publicly on a website maintained by the Department of Health and Human Services not later than 30 days after submission to Congress.

[All dollar amounts, percentage thresholds, time thresholds, and IRC section cross-references marked [TBD] require resolution by licensed legislative counsel, the Department of Health and Human Services Office of General Counsel, the Centers for Medicare and Medicaid Services Office of the Actuary, the Department of Defense Office of General Counsel, the Department of Justice Office of Legal Counsel, and the Congressional Budget Office before introduction as formal legislation. See Part III counsel notes. The framework deliberately defers operative thresholds to drafting closer to introduction per the Phase III "skeletal by design" architecture.]

Part III — Notes for Congressional Counsel & Key Decisions

All [TBD] items and constitutional questions below require resolution before introduction. UCT operates within settled constitutional doctrine — Medicare-style federal program; *NFIB v. Sebelius* sustained the ACA framework. The risk vector is political and implementation, not constitutional. The Sec 2 + Sec 401 Architecture A flagship entry below documents the Phase III postural anchor unique to this bill — light constitutional foundation in operative bill text, operative Phase III precondition compliance framework, no fallback ladder needed because constitutional architecture is settled. DOJ Office of Legal Counsel and CMS Office of the Actuary pre-introduction review required.

Item Requiring Resolution	Guidance
Sec 2 + Sec 401 — Architecture A Phase III postural anchor [NEW FLAGSHIP]	Architecture: Architecture A — federal program with body-baked Phase III precondition acknowledgment and Part III monitoring framework. UCT operates within settled constitutional doctrine: Medicare-style federal program; <i>NFIB v. Sebelius</i> sustained the ACA framework; Medicare has operated since 1965 without successful constitutional challenge. There is no novel constitutional risk to hedge. (1) Primary structure. Sec 2 light four-anchor constitutional foundation (Spending/Taxation Clause / Pennhurst clear-notice / coverage guarantee anti-taking / voluntary transition) anchors the architecture in operative bill text. Sec 401 Phase III Precondition Compliance is the operative innovation specific to Phase III bills — program operational status is conditioned on Secretary certification of each precondition. (2) Risk vector — political and implementation, not constitutional. The bill's "fail and damage the agenda if introduced early" warning is the central risk vector, not a Bruen-style or First Amendment doctrinal vulnerability. Sec 401(c) Congressional Review Act disapproval mechanism preserves congressional oversight at certification. (3) Bottom-level fallback. Permanent hybrid system: Public Option (Bill 6) and private plans coexist indefinitely. Stable, defensible endpoint if universal transition proves politically impossible. If Sec 401 certification cannot be achieved, this Act remains enacted but inoperative — Sec 401(e) "no operational status without certification" recital governs. (4) Pre-introduction engagement. DOJ Office of Legal Counsel review required. CMS Office of the Actuary modeling required for Sec 101(a) age-band expansion costs and Sec 202(a) provider rate transition. Senate Finance, HELP, and Armed Services Committee constitutional counsel pre-introduction briefing required. Phase III precondition checklist (Part III entry #10 below) must be satisfied before introduction.

Item Requiring Resolution	Guidance
Transition model selection	Three models for counsel to analyze: Canadian single-payer (provincial administration), German multi-payer regulated system (nonprofit insurers), or hybrid Medicare expansion. German model may face less constitutional challenge as it preserves regulated private insurers. Sec 402(a) requires the Secretary to complete international precedent review covering all three models within [TBD] months of enactment, and to report to Congress on the recommended transition model. The completed review is a Sec 401(a)(6) precondition for operational commencement.
Age-band sequencing rationale (Sec 101(a))	Starting with 55-64 maximizes political sympathy (pre-Medicare gap), minimizes actuarial cost per person, and creates a natural expansion pathway. Congressional Budget Office to model each age-band expansion for cost and coverage impact. The 55-64 band has the highest uninsured costs and greatest political sympathy — these features are essential to the bill's political viability and should not be modified absent strong evidence. Counsel should review Sec 101(a) timing (Year 1, 3, 5, 7, 9 from operational commencement) against CMS actuarial capacity assessment — the two-year gap between expansions provides time for absorption but may be too aggressive depending on system-strain modeling. Pre-introduction CMS Office of the Actuary review required.
Employer transition mechanism (Sec 101(c))	Employer transition payment must be set high enough to discourage mass immediate dumping of employees onto the public plan before the system can absorb them. Treasury and CMS actuarial review required. The Sec 101(c) [TBD] percent of foregone premium contributions for [TBD] years is the operative variable — these thresholds require pre-introduction calibration to balance employer voluntariness against system-absorption pacing. Pennhurst clear-notice requirement (Sec 2(b)) applies — operative conditions must be clearly stated in implementing regulations to permit employers to make informed compliance decisions. Counsel should review Sec 101(c) operative conditions against Pennhurst standards before finalizing implementing regulations.
Military budget reallocation (Sec 302) — most politically sensitive provision in the entire legislative ecosystem	This provision is the most politically sensitive in the entire legislative ecosystem. It should be structured as a proportional reallocation over 10-plus years, certified annually by the Joint Chiefs that readiness is maintained. DoD and Armed Services Committee consultation required before any draft. Sec 302 operative framework: proportional annual increments not to exceed [TBD] percent of base budget per fiscal year (Sec 302(a)), Joint Chiefs annual certification 90 days before fiscal year (Sec 302(b)), Senate Armed Services coordination with NDAA framework where NDAA controls in conflict (Sec 302(c)), fallback to other Sec 301(b) funding sources if certification denied (Sec 302(d)), explicit military readiness floor (Sec 302(e)). See Part III entry #6 below (Sec 302 Article I appropriations framework) for constitutional analysis. Pre-introduction DoD General Counsel and Senate Armed Services Committee constitutional counsel review required.

Item Requiring Resolution	Guidance
Sec 302 DoD reallocation — Article I appropriations framework [NEW]	Joint Chiefs certification (Sec 302(b)) and Senate Armed Services coordination (Sec 302(c)) operate within Congress's article I, section 8 power to "raise and support Armies" and the established appropriations-reallocation framework. While not strictly an NFIB v. Sebelius anti-coercion application (which addresses State conditioning rather than congressional appropriations), the structural concern is analogous: reallocation cannot be so substantial as to impair an essential constitutional function of the Federal government. The Joint Chiefs certification mechanism (Sec 302(b)) provides the structural safeguard, and the readiness floor (Sec 302(e)) is the explicit non-impairment recital. Sec 302(d) fallback if certification denied ensures that program operational capacity is preserved through other funding sources under Sec 301(b) when reallocation is suspended. Pre-introduction DoD General Counsel and Senate Armed Services Committee constitutional counsel review required. CBO scoring should explicitly model reallocation scenarios against military readiness assessments.
Provider rate transition period (Sec 202)	CMS to model minimum transition period needed to prevent provider exodus. Canadian transition took 10-plus years to stabilize. Recommend 7-year transition rate protection minimum. The Sec 202(a) [TBD] years and [TBD] percent of Medicare rates are the operative variables — these thresholds require pre-introduction calibration based on CMS actuarial modeling and provider network adequacy assessment. Counsel should review Sec 202(b) annual review and adjustment authority against potential rate-setting Penn Central regulatory takings exposure. Verizon Communications Inc. v. FCC, 535 U.S. 467 (2002), confirmed that rate regulation alone is not a per se taking but is evaluated under Penn Central balancing. The transition rate protection (above-Medicare rates) mitigates this exposure substantially.
Constitutional structure — Spending and Taxation Clause foundation [RECOVERED]	This bill is a Federal spending and taxation program — the strongest constitutional ground. The coverage guarantee and voluntary employer transition are essential to avoid "taking" arguments. Counsel to review Pennhurst and related cases. Sec 2 anchors this analysis in operative bill text: Sec 2(a) Spending and Taxation Clause authority (NFIB v. Sebelius / Medicare framework); Sec 2(b) Pennhurst clear-notice requirement for grant conditions; Sec 2(c) coverage guarantee anti-taking (Penn Central); Sec 2(d) voluntary transition. The constitutional architecture is settled doctrine — Medicare has operated since 1965 without successful constitutional challenge, and the ACA framework was sustained in NFIB v. Sebelius (2012). Pre-introduction DOJ Office of Legal Counsel review recommended to confirm operative provisions track each anchor.

Item Requiring Resolution	Guidance
Sec 401 Phase III Precondition Compliance mechanics [NEW]	Sec 401 establishes the operative certification framework: Secretary may not commence operational status until certifying each precondition under Sec 401(a)(1)–(6); Sec 401(b) congressional review through Senate Finance, HELP, Armed Services Committees and House comparable; Sec 401(c) Congressional Review Act disapproval mechanism within 90 days; Sec 401(d) withdrawal authority; Sec 401(e) no-operational-status-without-certification recital. Chadha review. Counsel should review Sec 401(c) joint-resolution-of-disapproval mechanism against <i>INS v. Chadha</i>, 462 U.S. 919 (1983), one-house-veto prohibition. CRA-style joint resolution requires both Houses plus presentment, which avoids Chadha. Sec 401(c) expressly recites the both-Houses-plus-presentment requirement and cross-references CRA procedure under chapter 8 of title 5, U.S.C. Operative effectiveness. Sec 401(e) recital is the strongest legal anchor — no provision of this Act is operationally effective without certification, except for planning, regulatory drafting, and interagency coordination provisions explicitly identified.
Political precondition checklist	Before introduction, counsel should obtain: (1) CBO preliminary score, (2) AMA and hospital association position, (3) Public Option performance data, (4) Senate vote count assessment. Do not introduce without 55-plus Senate votes identified. Operative cross-reference: the political precondition checklist tracks Sec 401(a) certification factors closely. Sec 401(a)(4) "the Senate has ratified, by joint resolution, that political conditions for introduction are met, or comparable political precondition has been documented" provides flexibility for either formal Senate ratification or documented political precondition equivalence. Counsel should preserve both pathways. See Phase III warning banner above (top of document) for the introduction admonition. The operative bill text and the warning banner together establish the framework: the bill must NOT be introduced before preconditions are met (banner), and the program must NOT commence operational status before Sec 401(a) certification (operative).

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Phase III — do not introduce before preconditions are met. Legal disclaimer applies — see page 1. This framework requires licensed legislative counsel review before introduction.

Originally conceived and authored by a concerned American citizen. Developed with the assistance of Claude (Anthropic). The ideas belong to anyone willing to fight for them.